DISCHARGE SUMMARY

Admitted: 09/27/2026 | Discharged: 10/02/2026 | Attending: Daniel K. Abernathy, MD, Orthopedic Surgery

Principal diagnosis

Displaced fracture of the right femoral neck (ICD-10 S72.001A).

Procedure

Right hip hemiarthroplasty, 09/28/2026.

Other diagnoses

Hypertension. Type 2 diabetes mellitus, hemoglobin A1c 7.1%. Osteoporosis.

Hospital course

71-year-old woman who fell in her kitchen on 09/27/2026 and could not bear weight. Imaging showed a displaced right femoral neck fracture. She had an uncomplicated right hip hemiarthroplasty on 09/28/2026. She was out of bed with physical therapy on the first day after surgery. Pain was controlled with scheduled acetaminophen and occasional oxycodone. Hemoglobin was stable at 10.4 g/dL. No delirium. The incision was clean and dry at discharge.

Discharge medications

- Enoxaparin 40 mg subcutaneously once daily for 28 days.
- Acetaminophen 1,000 mg three times daily.
- Oxycodone 5 mg every 6 hours as needed for severe pain, 5 days.
- Lisinopril 10 mg daily. Metformin 500 mg twice daily. Calcium with vitamin D daily.

Activity

Weight bearing as tolerated with a front-wheeled walker. Posterior hip precautions for 6 weeks.

Disposition and follow-up

Home with home health services (skilled nursing and physical therapy). Orthopedic clinic visit on 10/16/2026.

Electronically signed: Daniel K. Abernathy, MD, 10/02/2026 10:37

OPERATIVE REPORT

Date of surgery: 09/28/2026 | Surgeon: Daniel K. Abernathy, MD | Anesthesia: spinal

Preoperative and postoperative diagnosis

Displaced fracture of the right femoral neck.

Procedure

Right hip hemiarthroplasty with a cemented femoral stem and bipolar head.

Description

Posterior approach. The femoral head was removed and measured. The femoral canal was prepared and a cemented stem was placed. Trial reduction showed a stable hip with equal leg lengths. Final components were placed. The capsule and short external rotators were repaired and the wound was closed in layers.

Estimated blood loss and complications

250 mL. No complications. The patient went to recovery in stable condition.

Postoperative plan

Weight bearing as tolerated. Posterior hip precautions. Enoxaparin from the morning after surgery. Physical therapy from the first day after surgery.

Electronically signed: Daniel K. Abernathy, MD, 09/28/2026 13:55

LABORATORY RESULTS AND VITAL SIGNS

Reporting period: 09/27/2026 to 10/02/2026

09/27/2026, on admission

- Hemoglobin 12.8 g/dL. White cell count 9.6. Platelets 232.
- Sodium 138. Potassium 4.1. Creatinine 0.8 mg/dL. Glucose 164 mg/dL.
- INR 1.0. Hemoglobin A1c 7.1%.

09/29/2026, first day after surgery

- Hemoglobin 10.6 g/dL. Potassium 3.9. Creatinine 0.8 mg/dL.

10/01/2026

- Hemoglobin 10.4 g/dL. Potassium 4.0. Creatinine 0.8 mg/dL. Glucose 138 mg/dL.

Vital signs at discharge, 10/02/2026

BP 132/76, HR 78, temperature 36.8 C, oxygen saturation 97 percent on room air.

INPATIENT PHYSICAL THERAPY EVALUATION

Date: 09/30/2026 | Therapist: Jordan Feld, PT, DPT | Second day after surgery

Prior level of function

Walked in the community without a device. Lives with her daughter in a single-story house. Before this admission the patient was not homebound; she drove and shopped independently.

Current function

- Bed mobility: minimal assistance.
- Sit to stand: moderate assistance of one person.
- Walking: 40 feet with a front-wheeled walker and minimal assistance.
- Stairs: not attempted.
- Timed Up and Go: 38 seconds. Pain with activity: 5 out of 10.

Assessment

She requires skilled physical therapy for gait training, transfer training, hip precaution teaching, and progressive strengthening. Rehabilitation potential is good.

Recommendation

Physical therapy 5 times per week in a facility, or 3 times per week at home if a trained caregiver is present. Equipment: front-wheeled walker, raised toilet seat, shower chair.

Electronically signed: Jordan Feld, PT, DPT, 09/30/2026 15:20

FACE-TO-FACE ENCOUNTER AND HOME HEALTH ORDER

Physician: Daniel K. Abernathy, MD | Form completed: 10/01/2026

Face-to-face encounter

Encounter date: 10/01/2026. The encounter was related to the primary reason for home health: care after right hip hemiarthroplasty.

Orders and plan of care

I order home health services under this plan of care: skilled nursing 2 visits per week for 6 weeks (12 visits) and physical therapy 3 visits per week for 6 weeks (18 visits).

Skilled nursing need

Skilled nursing is needed for surgical incision assessment, enoxaparin injection teaching and monitoring, and pain medication management.

Skilled therapy need

Gait and transfer training, hip precautions, strengthening, and fall prevention, per the physical therapy evaluation of 09/30/2026.

Homebound status

Member is homebound: [ ] Yes [ ] No. Clinical findings supporting homebound status: (left blank).

Goals

Independent walking with a walker for 150 feet, independent transfers, a healed incision, and safe completion of enoxaparin by day 28.

Electronically signed: Daniel K. Abernathy, MD, 10/01/2026 14:12

CASE MANAGEMENT - DISCHARGE PLANNING NOTE

Date: 10/01/2026 | Case manager: Tamika Brooks, RN, CCM

Level of care

Patient meets Alderpoint criteria for skilled nursing facility admission: she requires daily skilled nursing and skilled physical therapy 5 times per week after hip hemiarthroplasty. Referral sent 09/30/2026. Bed offered at Cedar Hollow Rehabilitation and Nursing for 10/02/2026.

Patient and family preference

The patient and her daughter decline skilled nursing facility placement and ask for discharge home with home health services.

Caregiver

Her daughter, Anna Kowalczyk, lives with the patient, is available 24 hours a day for the first 6 weeks, and completed transfer and injection training on 10/01/2026.

Cost comparison

Expected cost at Plan contracted rates: skilled nursing facility, 20 days at $520 per day = $10,400; home health, 30 visits at $145 per visit = $4,350. Home health is the lower-cost option.

Equipment

Front-wheeled walker, raised toilet seat, and shower chair delivered to the home on 10/01/2026.

Plan

Discharge home on 10/02/2026 with home health. Prior authorization request sent to Alderpoint Health Plan on 10/01/2026.

Electronically signed: Tamika Brooks, RN, CCM, 10/01/2026 16:05

OCCUPATIONAL THERAPY - HOME SAFETY EVALUATION

Date: 10/01/2026 | Therapist: Wei Lin, OTR/L | Home visit before discharge, daughter present

Findings

- Single-story house. One step at the front entry with a handrail.
- Bedroom and bathroom on the main level. Doorways are wide enough for a walker.
- Grab bars installed in the shower on 09/30/2026. Night lighting in the hallway and bathroom.
- Throw rugs removed from the kitchen and bedroom. One rug remains in the hallway.

Conclusion

The home setting is clinically appropriate and safe for discharge with home health nursing and physical therapy.

Recommendations

Remove the remaining hallway rug. Use the shower chair and raised toilet seat. Keep the walker within reach at night.

Electronically signed: Wei Lin, OTR/L, 10/01/2026 13:40

ORTHOPEDIC CLINIC - TELEPHONE FOLLOW-UP AFTER DISCHARGE

Date: 10/06/2026 | Taken by: Carmen Delgado, RN

Spoke with the patient and her daughter. Pain is 4 out of 10 and controlled with acetaminophen. The daughter reports the incision is dry with no redness. Enoxaparin is given each morning by the daughter.

The patient says she has mostly been staying in since coming home. Her daughter drove her to the pharmacy on 10/05/2026; the patient waited in the car.

She walks to the bathroom and kitchen with the walker while her daughter stands by. No falls. Blood glucose readings 110 to 150 mg/dL.

Home health start of care is on hold pending insurance authorization. Reminded of the orthopedic clinic visit on 10/16/2026. Advised to call for fever, wound drainage, calf pain, or shortness of breath.

Electronically signed: Carmen Delgado, RN, 10/06/2026 11:18